

Clinical Brief

Patient ID: P003

Alert Summary

trigger: Weight gain of 3.1 kg over 14 days (from 86.2 kg to 89.3 kg), exceeding the threshold of 88.0 kg in a heart failure patient, detected by Withings Smart Scale.

severity_class: ELEVATED

severity_score: 6

timestamp: 2026-05-14T08:00:00Z

Patient Snapshot

patient_id: P003

demographics: Age and gender not available in EHR

active_conditions

- Heart Failure with reduced ejection fraction (active, since 2023)
- Type 2 Diabetes (active, since 2017)

current_medications

- Furosemide 40mg daily
- Carvedilol 12.5mg twice daily
- Sacubitril-Valsartan 49/51mg twice daily
- Spironolactone 25mg daily

contextual_analysis: The patient has a history of heart failure with reduced ejection fraction (EHR). Over the past two weeks, there has been a gradual weight gain of 3.1 kg, now 1.3 kg above the set threshold, with concurrent reports of moderate ankle swelling, mild fatigue, and reduced exercise tolerance (Anamnesis). Recent increased salt intake was noted around a social event, and the patient expresses concern about the weight gain and swelling (Anamnesis). Medication adherence to heart failure therapies is reported as good (Anamnesis).

risk_assessment: This pattern of weight gain with peripheral edema and mild symptoms may suggest early fluid overload in the context of heart failure. The absence of acute symptoms but presence of gradual deterioration could indicate a need for timely adjustment of diuretic therapy or further clinical evaluation.

recommended_actions

- action: Consider same-day clinical review to assess for fluid overload and evaluate need for diuretic adjustment.
confidence: 0.9
source: Alert + Anamnesis + EHR
- action: Review recent labs (BNP, renal function, electrolytes) and consider repeat testing if clinically indicated.
confidence: 0.85
source: EHR
- action: Reinforce dietary sodium restriction and monitor for further weight changes or worsening symptoms.
confidence: 0.8
source: Anamnesis

uncertainties_and_gaps

- Patient age and gender not available in EHR.
- No recent BNP, sodium, potassium, or creatinine labs since 2026-03-01.
- No recent documentation of ejection fraction or response to prior diuretic adjustment.
- No explicit documentation of orthopnea or paroxysmal nocturnal dyspnea.
- Fluid intake and restriction compliance not reported.
- Some anamnesis entries lack symptom and family history details.